National Cancer Control Programme (NCCP) 00890 · clinician draft

Fruquintinib (Fruzaqla) monotherapy

Previously treated colorectal cancer The team confirms the full treatment history, including chemotherapy, anti-VEGF and anti-EGFR treatment where applicable, and progression or intolerance on trifluridine/tipiracil or regorafenib.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Fruquintinib: Days 1–21, then Days 22–28 off

Active medicines: Fruquintinib

Emetogenic risk: Minimal-to-low oral

Medicine	Dose / route	When
Rescue antiemetic	Individual prescription under the oral-systemic anticancer therapy (SACT) pathway	As required; no routine daily prophylaxis NCCP classification V6 Table 5 provides metoclopramide, prochlorperazine or one 5-HT3 antagonist as options; choose rather than automatically combine, after interaction/ contraindication review.

Additional phase-specific support

- Premedication is not usually required. Use local skin-care guidance for hand-foot skin reactions and report painful, blistered or function-limiting symptoms early.
- Blood pressure weekly for the first month, then monthly and as clinically indicated. Keep prescribed antihypertensive treatment and report severe headache, visual change, chest pain or neurological symptoms urgently.
- Baseline and regular full blood count (FBC), renal/liver profile, urine dipstick for protein and thyroid function. international normalised ratio (blood-clotting test) (INR) if clinically indicated; if taking warfarin, weekly INR until a stable warfarin dose is established, then before each cycle.
- Review bleeding, gastrointestinal perforation, wounds and planned surgery; follow the separately prescribed interruption/restart plan. Do not borrow the regorafenib dose-modification table.
- Women of childbearing potential: highly effective contraception during treatment and for at least two weeks after the last dose, per NCCP.
- Confirm the prescribed daily dose and Days 1–21 calendar; dose reductions are consultant-led and treatment must stop if 3 mg daily is not tolerated.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00890 v2a · p2,4–5 · NCCP regimen; NCIS Medical Oncology antiemetics V8 Table 1

Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required